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## 環球健康 (UHS) 2026 Q1 法說會逐字稿

公布時間

26/04/28

本季實際 EPS

5.65 美元

EPS 比市場預期高

+6.81 %

EPS 年成長

+16.74 %

### 完整原文

使用警語：中文譯文來源為 AI 翻譯，僅供參考，實際內容請以英文原文為主

#### Operator

Good day, and thank you for standing by. Welcome to the first-quarter 2026 Universal Health Services earnings conference call. (Operator Instructions) Please be advised that today's conference is being recorded.

大家好，感謝您稍候。歡迎參加 Universal Health Services 2026 年第一季財報電話會議。(接線員指示) 請注意，今天的會議將被錄音。

I would now like to hand the conference over to your speaker today, Darren Lehigh, Vice President of Investor Relations. Please go ahead.

現在我想把會議交給今天的講者，投資人關係副總裁 Darren Lehigh。請開始。

#### Darren Lehigh - Vice President of Investor Relations

Thanks, Daniel. Good morning, and welcome to Universal Health Services first-quarter 2026 earnings conference call. I'm Darren Lehigh, Vice President of Investor Relations. With me this morning are our President and CEO, Marc Miller; and our Chief Financial Officer, Steve Filton. Marc and Steve will provide some prepared remarks, and then, we will open it up to Q&A.

謝謝你，Daniel。各位早安，歡迎參加 Universal Health Services 2026 年第一季財報電話會議。我是投資人關係副總裁 Darren Lehigh。今天早上與我一同出席的有我們的總裁兼執行長 Marc Miller，以及財務長 Steve Filton。Marc 與 Steve 將先做一些準備好的發言，接著我們將開放問答。

During today's conference call, we will be using words such as believes, expects, anticipates, estimate and similar words that represent forecasts, projections and forward-looking statements. For anyone not familiar with the risks and uncertainties inherent in these forward-looking statements, we recommend a careful reading of the section on risk factors and forward-looking statements and risk factors in our Form 10-K for the year ended December 31, 2025.

在今天的電話會議中，我們將使用例如 believes（相信）、expects（預期）、anticipates（預估）、estimate（估計）以及類似字詞，這些字詞代表預測、推估與前瞻性陳述。若您不熟悉這些前瞻性陳述所固有的風險與不確定性，我們建議您仔細閱讀我們截至 2025

年 12 月 31 日止年度 Form 10-K 中關於前瞻性陳述與風險因素的章節。

In addition, we may reference during today's call measures such as EBITDA, adjusted EBITDA, adjusted EBITDA net of NCI and adjusted net income attributable to UHS, which are non-GAAP financial measures. Information and reconciliations of these non-GAAP financial measures to net income attributable to UHS can be found in today's press release.

此外，在今天的電話會議中，我們可能會提及例如 EBITDA、調整後 EBITDA、扣除 NCI 的調整後 EBITDA，以及歸屬於 UHS 的調整後淨利等指標，這些均為非 GAAP 財務衡量指標。有關這些非 GAAP 財務衡量指標與歸屬於 UHS 的淨利之間的資訊與調節表，可見於今日的新聞稿。

With that, let me now turn it over to Marc Miller for some introductory remarks.

接下來，讓我把時間交給 Marc Miller，請他做一些開場說明。

### **Marc Miller - President, Chief Executive Officer, Director**

Thank you, Darren. Good morning to all participants on today's call, and thank you for your continued interest in UHS. The first quarter of 2026 feature significant acceleration in our behavioral health outpatient strategy with the announcement of the Talkspace acquisition and continued steady operating performance and cash flow generation in our core operations in the midst of a more challenging seasonal volume trends.

謝謝你，Darren。各位參與今天電話會議的朋友早安，感謝各位持續關注 UHS。2026 年第一季，我們的行為健康門診策略顯著加速，包含宣布收購 Talkspace；同時，在更具挑戰性的季節性就診量趨勢之下，我們核心營運仍維持穩健的營運表現與現金流創造。

Revenue growth for the first quarter was 9.6%. Adjusted EBITDA net of NCI increased 8.4% and adjusted EPS increased 16.1% as compared to the first quarter of 2025. These results highlight the adaptability and financial discipline of our leadership teams and the benefits of our efficiency initiatives, which are driven by technology adoption and operational excellence.

第一季營收成長 9.6%。扣除 NCI 的調整後 EBITDA 成長 8.4%，調整後 EPS 較 2025 年第一季成長 16.1%。這些結果凸顯我們領導團隊的適應力與財務紀律，以及效率提升計畫所帶來的效益；這些計畫由科技導入與卓越營運所驅動。

Speaking first to the Talkspace acquisition announced on March 9. Talkspace is an established market leader in virtual outpatient behavioral health care with a network of 6,000 licensed professionals serving all 50 states.

先談 3 月 9 日宣布的 Talkspace 收購案。Talkspace 是虛擬行為健康門診照護的既有市場領導者，擁有 6,000 名持照專業人士的網絡，服務涵蓋全美 50 州。

We believe Talkspace is the best-in-class virtual platform in the behavioral industry with differentiated technology offering and strong brand recognition among patients and clinicians. Talkspace's successful payer-driven business model aligns well with our strategy to increase access to a full spectrum of outpatient services and diversify our behavioral payer mix.

我們相信 Talkspace 是行為健康產業中同級最佳的虛擬平台，具備差異化的技術能力，並在病患與臨床人員之間擁有高度品牌辨識度。Talkspace 以支付方驅動的商業模式運作成功，與我們提升全方位門診服務可近性、並使行為健康支付方組合更多元化的策略高度契合。

Over the past 24 months, we focused significant resources to grow existing outpatient service locations adjacent to our hospital campuses and develop new freestanding outpatient clinic locations. We will continue to invest in these areas internally. The addition of Talkspace's high-quality scaled platform accelerates our ability to create the industry's first end-to-end continuum of behavioral health care services that is strongly aligned to the demand trends and preferences of the market overall.

過去 24 個月，我們投入大量資源，擴增鄰近醫院院區的既有門診服務據點，並開發新的獨立式門診診所據點。我們將持續在內部投資這些領域。加入 Talkspace 這個高品質且具規模的平台，將加速我們打造業界首個端到端行為健康照護服務連續體的能力，並與

整體市場的需求趨勢與偏好高度一致。

This national continuum includes lower acuity outpatient and step-in services all the way to residential and inpatient services, where we've led the market for more than four decades. We plan to share more details about the impact of the transaction after closing, but I'd like to highlight two primary benefits for the acquisition.

這個全國性的連續體涵蓋較低急性度的門診與銜接服務，一直到住宿式與住院服務；而在後者領域，我們已領先市場超過四十年。我們計畫在交易完成後分享更多關於交易影響的細節，但我想先強調此次收購的兩項主要效益。

First, from a strategic perspective, Talkspace represents a multi-year value creation opportunity, underpinned by access to new sources of outpatient revenue growth. This is supported first by the strength of the base, Talkspace business, which has a very strong outlook on a stand-alone basis and enhanced further by the programs we plan to develop alongside Talkspace to complement each other's businesses.

第一，從策略角度來看，Talkspace 代表一個為期多年的價值創造機會，其基礎在於取得新的門診營收成長來源。這首先來自 Talkspace 既有業務基礎的強勁實力；其單獨運作的前景非常正向，並將因我們計畫與 Talkspace 共同開發、以互補彼此業務的方案而進一步增強。

For example, there is a significant opportunity for us to introduce Talkspace's 6,000 clinicians into our environment to develop higher acuity virtual offerings such as virtual intensive outpatient programs or IOPs. This will improve our ability to manage more patients stepping down from UHS facilities with a preferred virtual option.

例如，我們有很大的機會將 Talkspace 的 6,000 名臨床人員導入我們的體系，開發更高急性度的虛擬服務，例如虛擬密集門診計畫（virtual intensive outpatient programs，或 IOPs）。這將提升我們管理更多自 UHS 機構轉銜下來之病患的能力，並提供其偏好的虛擬選項。

The types of programs we build on a virtual outpatient basis will drive higher-quality continuity of care further downstream after our patients step down from higher levels of care. There are numerous other bidirectional revenue synergy opportunities we'll be working on post-closing that will improve access to outpatient virtual services for UHS patients and improve access to higher levels of care for Talkspace patients.

我們以虛擬門診形式建立的各類方案，將在病患自較高照護層級轉銜後，於後續流程中帶來更高品質的照護連續性。在交易完成後，我們將推動許多其他雙向的營收協同機會，以提升 UHS 病患取得門診虛擬服務的可近性，並提升 Talkspace 病患取得更高照護層級服務的可近性。

Second, from a financial perspective, we expect the deal to be accretive to earnings during the first 12 months post-closing, and we expect it to be increasingly accretive thereafter. By year three post-closing, we expect the effective EBITDA multiple for the Talkspace transaction to be in the single-digit range.

第二，從財務角度來看，我們預期此交易在交割後的前 12 個月將對盈餘具增益效果，且之後增益效果將逐步提高。在交割後第三年，我們預期 Talkspace 交易的有效 EBITDA 倍數將落在個位數區間。

Moving on to the quarter. I'd like to highlight a few items before I turn it over to Steve to review the financials. From a growth perspective, we met our internal same-facility revenue growth and earnings objectives in the first quarter despite a more dynamic operating backdrop.

接著談本季表現。在我把時間交給 Steve 檢視財務之前，我想先強調幾點。從成長角度來看，儘管營運環境更為多變，我們第一季仍達成內部設定的同院營收成長與獲利目標。

This was accomplished through solid expense management and higher contributions from pricing in both segments due to more positive trends in rate. We expect same-facility growth to be more balanced between volume and pricing as the year progresses, as

we believe first quarter volume performance was impacted heavily by seasonal factors consistent with what we highlighted in February on our fourth quarter earnings call.

這主要透過穩健的費用管理，以及兩大事業部門因費率趨勢較為正向而帶來的較高定價貢獻所達成。隨著年度推進，我們預期同院成長在量與價之間將更為均衡；因為我們認為第一季的量表現受到季節性因素的顯著影響，這與我們在 2 月第四季財報電話會議中所提到的情況一致。

From a technology perspective, our enterprise-level AI governance process remains very active and focused on two primary domains within our business, in the operational domain to impact quality and patient experience and in the administrative domain to increase efficiency.

從科技角度來看，我們企業層級的 AI 治理流程仍非常活躍，並聚焦於業務中的兩個主要領域：營運領域，用以影響品質與病患體驗；以及行政領域，用以提升效率。

During 2025, we focused heavily on scaling solutions that reduce the burden of our routine administrative tasks. We deployed and scaled a total of eight different use cases of AI solutions into our revenue cycle operations that are now yielding significant benefit on a go-forward basis.

在 2025 年，我們高度聚焦於擴大可降低例行行政工作負擔的解決方案。我們在營收循環作業中部署並擴大了共八項不同的 AI 解決方案使用情境，這些方案目前正持續帶來顯著效益。

For 2026, we are focusing more heavily on enabling solutions in our clinical operations to improve hospital-level efficiency and patient experience. Included in our 2026 road map are several new use cases being designed and built with Hippocratic AI, which is one of our key AI solution partners.

在 2026 年，我們將更著重於在臨床營運中導入賦能型解決方案，以提升醫院層級效率與病患體驗。我們 2026 年的路線圖包含數個新的使用情境，正與 Hippocratic AI（我們重要的 AI 解決方案合作夥伴之一）共同設計與建置。

It is too early to project the longer-term financial impact of the 2026 initiatives, although we expect them to be incremental to margins over time. And just as importantly, we expect them to have a real impact on quality and patient experience.

目前仍言之過早，尚無法預估 2026 年各項計畫對長期財務的影響；不過我們預期，隨時間推進，這些計畫將對利潤率帶來增量貢獻。同樣重要的是，我們預期它們將對品質與病患體驗產生實質影響。

In closing, I am encouraged by our progress so far in 2026 and remain optimistic about our ability to deliver high-quality services in an efficient manner in the communities we serve.

最後，我對我們在 2026 年迄今的進展感到振奮，並對我們能以高效率的方式在所服務的社區提供高品質服務，持續保持樂觀。

On behalf of our entire organization, we look forward to welcoming Talkspace employees into UHS in the coming months.

我謹代表整個組織，期待在未來幾個月歡迎 Talkspace 的員工加入 UHS。

With that, I will now turn the call over to Steve Filton for more details on the quarter.

接下來，我將把電話交給 Steve Filton，請他就本季提供更多細節。

### **Steve Filton - Chief Financial Officer, Executive Vice President, Secretary**

Thanks, Marc. I'll highlight a few financial and operational trends before opening the call up to questions. The company reported net income attributable to UHS per diluted share of \$5.65 for the first quarter of 2026. After adjusting for the impact of the items reflected on the supplemental schedule, as included with the press release, our adjusted EPS was \$5.62 for the first quarter.

謝謝你，Marc。在開放提問之前，我先重點說明幾項財務與營運趨勢。公司於 2026 年第一季報告之歸屬於 UHS 的每股稀釋後淨利為 5.65 美元。在就新聞稿所附補充明細表中所反映項目的影響進行調整後，我們 2026 年第一季的調整後 EPS 為 5.62 美元。

On a same-facility basis, adjusted admissions at our acute care hospitals were unchanged as compared to the first quarter of 2025. We estimate acute care volumes during the first quarter of 2026 were impacted by approximately 200 basis points due to weaker flu and respiratory activity and winter weather in certain markets.

以同院（same-facility）基礎計算，我們急性照護醫院的調整後入院人次與 2025 年第一季相比持平。我們估計 2026 年第一季的急性照護量因流感與呼吸道活動較弱，以及部分市場的冬季天候影響，約受到 200 個基點的衝擊。

Performance in the Nevada market rebounded slightly with adjusted admissions increasing approximately 1.5% over the prior year. Same-facility acute care emergency department visits increased approximately by 2%, and we also saw positive trends in certain higher acuity important service lines or inpatient service lines, notably cardiology, orthopedics and neurology.

內華達市場的表現略有回升，調整後入院人次較去年增加約 1.5%。同院急性照護急診就診人次約增加 2%，我們也在某些更高病情嚴重度的重要服務線或住院服務線看到正向趨勢，尤其是心臟科、骨科與神經科。

On a same-facility basis, net revenues in our acute care hospital segment during the first quarter of 2026 increased 8.2% and were up 6.2% excluding the impact of our health plan. Acute care same-facility revenue per adjusted admission increased 6.3% during the first quarter of 2026 on a reported basis and was up 4.9% after excluding approximately \$30 million of prior period supplemental program net benefit related to the expanded 2025 Nevada program, which we contemplated in our guidance.

以同院基礎計算，2026 年第一季我們急性照護醫院事業部的淨營收增加 8.2%；若排除健康保險計畫的影響，則增加 6.2%。2026 年第一季急性照護同院每調整後入院人次營收（revenue per adjusted admission）在申報口徑下增加 6.3%；若再排除約 3,000 萬美元、與擴大 2025 年內華達計畫相關且屬前期補充方案（supplemental program）之淨收益（我們已在指引中納入考量），則增加 4.9%。

Operating expenses were well managed across labor, supply and other expense categories. Same-facility acute care salaries, wages and benefits expense per adjusted admission increased 3.1% and supply expense per adjusted admission increased 3.5% over last year's first quarter.

營運費用在勞動力、供應與其他費用類別方面均管理得宜。同院急性照護每調整後入院人次的薪資、工資與福利費用較去年第一季增加 3.1%，每調整後入院人次的供應費用增加 3.5%。

Same-facility contract labor was 2.3% of Acute Care segment revenues or 40 basis points lower year-over-year. Other operating expenses increased primarily as a result of the growth in our health plan. For the first quarter of 2026, our acute care performance resulted in 11.7% growth in same-facility segment EBITDA.

同院合約勞務成本占急性照護事業部營收的 2.3%，年減 40 個基點。其他營運費用的增加主要源自我們健康保險計畫的成長。2026 年第一季，我們的急性照護表現帶動同院事業部 EBITDA 成長 11.7%。

Excluding the prior period supplemental program revenue first quarter 2026 same-facility acute care segment revenue would have increased 3.3% on a year-over-year basis. With respect to health insurance exchange trends during the first quarter of 2026, we estimate an impact of approximately \$15 million.

若排除前期補充方案收入，2026 年第一季同院急性照護事業部營收年增將為 3.3%。就 2026 年第一季健康保險交易所（exchange）趨勢而言，我們估計影響約為 1,500 萬美元。

Our exchange adjusted admissions declined approximately 5% as compared to the first quarter of 2025. However, due to our expectation that some of the exchange members treated at our acute care facilities during the first quarter will not sustain their premium payments, the impact to our acute care financials assumes an effective HICS decline that is higher than the reported trend.

We are reiterating the full year \$75 million pre-tax impact, which assumes the exchange declines will steepen somewhat as the year progresses.

我們的交易所 (exchange) 調整後入院人次較 2025 年第一季約下降 5%。然而，由於我們預期第一季在急性照護院所接受治療的部分交易所會員將無法持續繳納保費，對急性照護財務的影響假設有效的 HICS 下滑幅度高於所呈報的趨勢。我們重申全年稅前影響 7,500 萬美元的預估，該預估假設交易所下滑將隨年度推進而略為加劇。

Turning to our Behavioral Health segment results during the first quarter of 2026. Same-facility net revenues increased 7.3%, supported by a 5.8% increase in same-facility revenue per adjusted patient day and a 1.6% increase in same-facility adjusted patient days as compared to the first quarter of 2025.

接著看 2026 年第一季行為健康 (Behavioral Health) 事業部的表現。同院淨營收增加 7.3%，主要受同院每調整後病人日 (revenue per adjusted patient day) 增加 5.8% 以及同院調整後病人日增加 1.6% (相較 2025 年第一季) 所支撐。

We estimate that the winter weather impacted first quarter behavioral health volume growth by approximately 40 to 50 basis points. Same-facility behavioral health segment EBITDA increased by 8.4% in the first quarter of 2026. Excluding the net benefit from prior period supplemental payments, same-facility revenue per adjusted patient day would have increased 4.9% and same-facility's segment EBITDA would have increased 4.3%.

我們估計冬季天候使第一季行為健康量的成長約受到 40 至 50 個基點的影響。2026 年第一季，同院行為健康事業部 EBITDA 增加 8.4%。若排除前期補充付款 (supplemental payments) 的淨收益，同院每調整後病人日營收將增加 4.9%，同院事業部 EBITDA 將增加 4.3%。

For the first quarter of 2026, Behavioral Health segment same-facility salaries, wages and benefits per adjusted patient day increased by approximately 6% on a year-over-year basis moderating slightly from the 7% to 8% level we experienced during 2025.

2026 年第一季，行為健康事業部同院每調整後病人日的薪資、工資與福利年增約 6%，較 2025 年我們經歷的 7% 至 8% 水準略有趨緩。

In California, we are making good progress year-to-date with respect to the state's nurse staffing ratio requirements that go into effect June 1, and we remain on track with the assumptions contemplated in our 2026 outlook.

在加州，針對將於 6 月 1 日生效的州政府護理人力配置比例要求，我們今年迄今已取得良好進展，且仍符合我們 2026 年展望中所假設的前提。

Cash generated from operating activities was \$402 million for the three months ended March 31, 2026, as compared to \$360 million during the same period last year. During the first quarter of 2026, we spent \$217 million on capital expenditures.

截至 2026 年 3 月 31 日止三個月，營運活動所產生的現金為 4.02 億美元，去年同期為 3.60 億美元。2026 年第一季，我們資本支出為 2.17 億美元。

In the Acute Care segment, we continue to invest in the 156-bed de novo hospital in Florida, scheduled to open in May, and in two bed towers and a replacement hospital project together comprising 178 beds that go online during the second quarter.

在急性照護事業部，我們持續投資佛羅里達州一所新建 (de novo) 的 156 床醫院，預計於 5 月開幕；並投資兩座病床大樓以及一項替換醫院 (replacement hospital) 專案，合計 178 床，將於第二季投入營運。

In our Behavioral Health segment, we opened a 144-bed de novo joint venture hospital in Pennsylvania in the early part of the first quarter and plan to open a 120-bed de novo hospital in Missouri later this year. During the first quarter of 2026, we acquired 675,000 of our shares at a total cost of \$127 million.

在行為健康事業部，我們於第一季初在賓州開設一所 144 床的新建（de novo）合資醫院，並計畫於今年稍晚在密蘇里州開設一所 120 床的新建醫院。2026 年第一季，我們以總成本 1.27 億美元回購 67.5 萬股自家股票。

As of March 31, 2026, we had \$1.3 billion of repurchase authorization available pursuant to our stock buyback program, and we expect to remain active with share repurchase throughout 2026, including leading up to and following the closing of the Talkspace acquisition.

截至 2026 年 3 月 31 日，依據我們的庫藏股回購計畫，尚有 13 億美元的回購授權額度可用；我們預期 2026 年將持續積極進行股票回購，包括在 Talkspace 收購案交割前後。

From a balance sheet perspective, in late April, we expanded the aggregate capacity of our credit facilities by \$900 million to provide additional flexibility with the pending Talkspace transaction, other potential acquisitions and our continued prioritization of returning capital to shareholders through buybacks and dividends.

就資產負債表而言，我們於 4 月下旬將信用融資額度的總容量擴增 9 億美元，以便在即將進行的 Talkspace 交易、其他潛在收購，以及我們持續透過回購與股利向股東返還資本的優先策略下，提供更大的彈性。

As of March 31, 2026, we have \$373 million of borrowings outstanding pursuant to our revolving credit facility, the borrowing capacity of which was recently expanded to \$1.5 billion.

截至 2026 年 3 月 31 日，我們依循環信用額度（revolving credit facility）之借款餘額為 3.73 億美元；該額度的借款上限近期已擴增至 15 億美元。

Turning to our outlook for 2026. We are reiterating the financial and operating forecasts that we established on February 25 in conjunction with fourth quarter earnings. Customary with our historical practice, we plan to reevaluate annual guidance as necessary in conjunction with our second quarter earnings planned for July.

接著談 2026 年展望。我們重申 2 月 25 日在第四季財報發布時所建立的財務與營運預測。依照我們一貫的慣例，我們計畫在 7 月預定發布的第二季財報時，視需要重新評估年度指引。

Operator, that concludes our prepared remarks, and we're pleased to answer questions at this time.

接線員，我們的準備發言到此結束，現在很樂意回答問題。

## Operator

A.J. Rice, UBS.

A.J. Rice，瑞銀（UBS）。

## AJ Rice - Analyst

I appreciate the number you gave on the behavioral side of 4.3%, sort of the normalized core growth. I'm wondering if I could ask you that maybe a lot of moving parts in the acute business, on the negative, you had the impact from the weather and the flu, and on the positive, DPP variance year-to-year. Can you sort of parse those out and give us a sense of what the core grew on an EBITDA basis in the acute side, possibly?

我很感謝你們在行為健康端提供的 4.3% 這個數字，算是常態化的核心成長。我想請教的是，急性照護業務這邊可能有很多變動因素；負面方面有天候與流感的影響，正面方面則有 DPP 年對年差異。你們能否把這些因素拆解一下，並讓我們了解急性照護端在 EBITDA 基礎上的核心成長大概是多少？

**Steve Filton - Chief Financial Officer, Executive Vice President, Secretary**

Yes. I think it was in the low single-digit range, A J.

是的。我認為是在個位數的低檔區間，A.J.。

**AJ Rice - Analyst**

Okay. All right. And then, obviously, interesting to talk about the AI use cases that Marc called out. And then, it sounds like there's some additional ones you're looking at. I wondered if you could pick out two or three that you're saying these are really meaningful and maybe delve in a little bit more on what you're seeing as an opportunity to deploy AI.

好的。好。接著，當然，很有意思的是談談 Marc 提到的 AI 使用案例。另外，聽起來你們也在看一些額外的案例。我想請你挑出兩到三個你認為真正很有意義的，並且可以再深入一點說明你看到部署 AI 的機會在哪裡。

**Marc Miller - President, Chief Executive Officer, Director**

Yes. I mean, as I said, A.J., we're looking to do things that affect our administrative functions to increase efficiency and in clinical operations, so if we can impact patient experience, improve outcomes. So that's kind of where our focus has been. We're already seeing some of those things paying dividends. For example, right now, one of the things on the operating side, we've deployed and scaled a total of eight different use cases of AI solutions into our revenue cycle operations.

是的。我的意思是，如我所說，A.J.，我們希望做一些能影響我們行政職能、提升效率的事情，也希望在臨床營運上有所助益，因此如果我們能影響病患體驗、改善結果。所以這大概就是我們的重點所在。我們已經看到其中一些開始帶來成效。例如，目前在營運端，我們已在營收循環作業中部署並擴大到總共八個不同的 AI 解決方案使用案例。

And we've already seen, like I said, significant benefits already, and I think this is going to continue and multiply going forward. So I think our efficiency is getting better, but we're also seeing it on the financial side in increased dollars, improvements with denials management and then hopefully increase revenue.

而且如我所說，我們已經看到相當顯著的效益，我認為未來這會持續並且倍增。所以我認為我們的效率正在改善，但我們也在財務面看到成效，包括金額增加、拒付管理的改善，並希望進一步提升營收。

And then, we're also doing a lot of things that are touching the patient. So anything that we can do in those two areas are kind of where we're going to focus for now. We're not doing much in the clinical space yet. But I do think down the line, we'll see something there as well.

另外，我們也在做很多直接觸及病患的事情。所以凡是能在這兩個領域有所作為的，將是我們目前的重點。我們在臨床領域目前還做得不多。但我確實認為長期來看，那裡也會看到一些成果。

**Operator**

Jason Cassorla, Guggenheim.

Jason Cassorla, Guggenheim。

**Jason Cassorla - Equity Analyst**

I wanted to check back in with the \$46 million of combined Nevada and Ohio out-of-period Medicaid supplemental payments. I think we're counting somewhere around \$120 million to \$130 million year-over-year benefit from total Medicaid supplemental payments in the quarter. Is that a fair characterization?



我想再確認一下內華達州與俄亥俄州合計 4,600 萬美元的跨期 Medicaid 補充付款。我想我們在本季的 Medicaid 補充付款總額上，年對年效益大約落在 1.2 億到 1.3 億美元左右。這樣描述合理嗎？

And then, if it's so, then could you help us bridge what would indicate a decent step-up in core EBITDA ramp for the remainder of the year to meet that 5% growth expectation? Just any color there would be helpful to start.

另外，如果是這樣的話，你能否協助我們銜接一下：為了達到你們所預期的 5% 成長，接下來一年其餘期間核心 EBITDA 的爬坡，哪些因素顯示會有不錯的提升？任何補充說明都會很有幫助。

### Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

Sure, Jason. So I think that's accurate. And I also think it's worth noting that nothing that you enumerated was outside of our expectations. Everything that you suggested, the -- all the DPP that we recorded or the vast majority of it in Q1 was in our guidance. I think that if you exclude the \$46 million of out-of-period DPP in Q1, you'll have a good run rate for the rest of the year, and that number is consistent with what we disclosed in our 10-K and will disclose in our first quarter 10-Q as our estimated DPP for the year.

當然，Jason。我認為這是正確的。我也認為值得注意的是，你所列舉的內容都沒有超出我們的預期。你提到的所有事項——我們在第一季認列的所有 DPP，或其中絕大多數——都已包含在我們的財測指引之中。我認為，如果你把第一季 4,600 萬美元的跨期 DPP 排除掉，你就會得到今年其餘期間一個不錯的常態化水準，而該數字也與我們在 10-K 中揭露、並將在第一季 10-Q 中揭露的全年 DPP 估計一致。

So we recognize that we would have this significant benefit in Q1, largely because we had a number of large DPP programs last year, if you could recall, Tennessee, D.C., that were not approved, and therefore, recorded and recognized until after the first quarter.

因此我們知道第一季會有這項顯著的效益，主要是因為去年有幾個大型 DPP 計畫——如果你還記得，田納西州、華盛頓特區——未獲核准，因此直到第一季之後才入帳並認列。

As far as the ramp for the rest of the year -- so I'm just trying to make the point that everything we recorded in Q1 was within our expectations. I think our overall results were within our expectations. And I think that implies that we expect a ramp in our earnings as the year goes on to get to that core level growth of 5% that's embedded in our guidance. That -- those assumptions include the continued ramp-up of the new facilities: Cedar Hill in Washington, D.C., which celebrated its first year anniversary this month, the opening of a new hospital in Florida, the opening, as I -- as we alluded to in our remarks, of 178 new beds in existing hospitals in California, in Las Vegas and in Florida.

至於今年其餘期間的爬坡——我只是想強調，我們在第一季認列的所有內容都在預期之內。我認為我們的整體結果也在預期之內。而這也意味著，隨著年度推進，我們預期獲利會逐步爬升，以達到財測指引中所隱含的 5% 核心成長。這些假設包括新設施的持續爬坡：位於華盛頓特區的 Cedar Hill，本月剛慶祝開業滿一週年；佛羅里達州一家新醫院的開幕；以及如我們在談話中提到的，在加州、拉斯維加斯與佛羅里達州的既有醫院新增 178 張病床的啟用。

The continued improvement in behavioral, both in outpatient revenues, higher-margin outpatient revenue business and continued operating leverage from growth in volumes. And I think volumes were on the softer side in both acute and behavioral, and we expect them both to improve as the year goes on.

行為健康業務的持續改善，包括門診營收（較高毛利的門診營收業務）的提升，以及因量能成長帶來的持續營運槓桿。我認為急性照護與行為健康的量都偏軟，而我們預期兩者都會在今年進行中逐步改善。

And -- finally, I think, as I also, I think, suggested in my remarks, we expect continued moderation in wage pressures in the behavioral business. We saw a significant investment in wages in behavioral in 2025, moderated a little bit in Q1 '26, but expected to moderate further as '26 progresses.

最後，我想如同我在談話中也提到的，我們預期行為健康業務的薪資壓力會持續趨緩。我們在 2025 年對行為健康的薪資做了相當

大的投資，到了 2026 年第一季稍有緩和，但預期隨著 2026 年推進會進一步緩和。

### Jason Cassorla - Equity Analyst

Got it. Great. Very helpful. And if I could just follow up maybe on the behavioral volume picture, excluding weather impacts, you still -- you hit the low end of your 2% to 3% volume target in the quarter. Maybe can you just talk about -- or talk a little bit more about the drivers of growth in the quarter?

了解。很好。非常有幫助。如果可以的話，我想再追問一下行為健康的量能情況：排除天氣影響，你們本季仍達到 2% 到 3% 量能目標區間的低端。也許你能談談——或再多談一點——本季成長的驅動因素？

Maybe how much of that kind of step up or that volume acceleration ex-weather was a function of the higher headcount and increased labor supply? Or are you seeing anything different in the demand picture or throughput there? Just any thoughts around the volume environment for behavioral would be helpful.

例如，這種提升或排除天氣後的量能加速，有多少是因為人力編制增加、勞動供給提升所致？或者你們在需求面或周轉效率（throughput）上看到任何不同的變化嗎？任何關於行為健康量能環境的看法都會很有幫助。

### Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

Sure. I think there's two broad trends. And I think you touched on at least one of them, Jason. Again, we made the point in our year-end call that we invested heavily in beefing up our staffing in behavioral in 2025. And the point of that was to allow us greater flexibility and ability to take on greater patient demand. And I think that's beginning to affect itself or reflect itself.

當然。我認為有兩個主要趨勢。而且我想你至少提到了其中之一，Jason。同樣地，我們在年終電話會議中提到，2025 年我們大幅投資以強化行為健康的人力配置。其目的在於讓我們有更大的彈性與能力去承接更高的病患需求。我認為這開始產生影響，或開始反映出來。

And then also, we've talked at great length over several quarters about our continued focus on outpatient growth in a business where we're finding more and more of the demand. And we think demand remains strong for behavioral services, but more and more of that demand is shifting to the outpatient setting. And I think we're doing more -- a better job and a more focused job on capturing that demand. And again, I think we think that's an upward climbing trajectory, as I alluded to in my answer to your first question.

另外，我們在過去幾季也長篇討論過，我們持續聚焦於門診成長，因為我們發現需求越來越多出現在這個領域。我們認為行為健康服務的需求仍然強勁，但越來越多的需求正在轉向門診場域。我認為我們在掌握這些需求方面做得更多、更好，也更聚焦。同樣地，我們認為這是一條向上攀升的軌跡，正如我在回答你第一個問題時所提到的。

### Operator

Ann Hynes, Mizuho.

Ann Hynes，Mizuho。

### Ann Hynes - Analyst

Can we just talk about bad debt reserve trends? I know this is a dynamic environment with some Medicaid disenrollment and also the expiration of the ACA subsidies. How is that trending versus your expectation? And can you remind us, does your guidance assume a deterioration of collectability on the co-pays to deductibles?

我們可以談談呆帳準備的趨勢嗎？我知道在 Medicaid 退保以及 ACA 補貼到期的情況下，這是一個變動的環境。目前的趨勢相較於你們的預期如何？另外也請提醒我們，你們的財測指引是否假設自付額與共同負擔（co-pays to deductibles）的可收回性會惡化？

**Steve Filton - Chief Financial Officer, Executive Vice President, Secretary**

Yes. Ann, so we addressed, I think, specifically kind of the HICS dynamic, as it relates to uncompensated care and bad debt in our prepared remarks. We saw a decline in HICS volume in Q1. But as I noted, we also recorded an additional reserve because we believe that some of those HICS patients who are presenting themselves as having HICS coverage will later be deemed not to have coverage because they fail to make their premium payments.

是的。Ann，我想我們在事先準備的講稿中，已特別針對 HICS 的動態——就無償照護與呆帳而言——做了說明。我們在第一季看到 HICS 量下降。但如我所提到的，我們也額外提列了一筆準備，因為我們認為部分自稱具有 HICS 保險覆蓋的病患，之後會被認為其實沒有覆蓋，原因是他們未能繳納保費。

So I think we've taken a reasonably conservative position in Q1. We continue -- we'll continue to learn more about those HICS dynamics. But at the moment, we believe that our \$75 million negative estimate for the impact of the HICS subsidies expiring is good. It will continue to get larger as the year goes on, but that was always our expectation.

所以我認為我們在第一季採取了相當保守的立場。我們會持續——我們將持續更深入了解那些 HICS 的動態。但就目前而言，我們相信對 HICS 補助到期影響的 7,500 萬美元負面估計是合理的。隨著年度推進，這個影響會持續擴大，但那一直都在我們的預期之內。

And that impact largely is reflected in higher levels of bad debt and uncompensated care. Other than that, I can't say that we saw any dramatic changes in our payer mix, slight increases in uninsured and Medicaid -- excuse me, slight -- yes, slight increases in uninsured, decreases in Medicaid utilization, slightly increase in Medicare utilization slightly, but no other big changes. No big changes in denials or patient status changes from the payers.

而這項影響主要反映在較高水準的壞帳與無償照護上。除此之外，我不能說我們在付款方組合上看到任何劇烈變化；無保險與 Medicare 略有增加、Medicaid 使用率略有下降——抱歉，略——是的，無保險略有增加、Medicaid 使用率下降、Medicare 使用率也略有增加，但沒有其他重大變化。在拒付或付款方對病人身分變更方面也沒有重大變化。

And I think we attribute a lot of that to the investments that Marc alluded to in technology and also in people and process in our revenue cycle. We've really been focused, particularly in the acute care segment on improvements in our revenue cycle efficiency.

我認為其中很大一部分要歸因於 Marc 提到的在科技，以及人員與流程（收入循環）方面的投資。我們一直非常專注於提升收入循環效率，尤其是在急性照護部門。

And I think that's allowing us to, at a minimum, keep pace with the payers, as they demonstrate more aggressive behavior. And as we've disclosed before, we're going to shift that focus or increase that focus to the behavioral segment in 2026 and our revenue cycle efficiencies in that segment.

我認為這至少讓我們能跟上付款方在行為上更為強勢的步伐。而且如同我們先前揭露的，我們將在 2026 年把這個重點轉移——或加大重點——到行為健康部門，以及該部門的收入循環效率。

**Operator**

Matthew Gillmor, KeyBanc.

Matthew Gillmor, KeyBanc。

**Matthew Gillmor - Equity Analyst**

I wanted to follow up on the pricing comments. I think Marc had mentioned the positive contribution in rate, but expecting a more balanced outlook for the balance of the year. It seemed like that outperformance was above and beyond your expectations even

excluding the SPP. So could you give us a sense for what drove the stronger pricing in the quarter? And what's behind your expectation for the moderation for the rest of the year?

我想追問一下定價方面的評論。我記得 Marc 提到費率 (rate) 有正向貢獻，但預期今年剩餘期間的展望會更為均衡。看起來即使排除 SPP，那個超預期表現也高於你們原先的預期。所以你們能否說明一下，本季較強的定價是由什麼驅動的？以及你們預期今年其餘時間會趨緩的原因是什麼？

### **Steve Filton - Chief Financial Officer, Executive Vice President, Secretary**

Well, I think a little bit of it, Matt, is simply the mix with a lower sort of flu component, a significantly lower flu component this year rather than last year. By definition, the patients that we had this year were of a higher acuity. Most flu and respiratory patients obviously carry a fairly low acuity.

我想其中一部分，Matt，純粹是組合因素：今年流感相關的占比更低——相較去年顯著更低。按定義，今年我們收治的病人病情嚴重度 (acuity) 更高。多數流感與呼吸道病患顯然屬於相對較低的嚴重度。

So I think that's a big piece of it. But I did stress, I think in -- or one of us stressed in our prepared comments that we also saw reasonably healthy increases in some of the more acute service lines, including cardiology, orthopedics and neurology. So that helped as well with the acuity and the pricing in acute care.

所以我認為這是很大的一塊。但我也強調過——我想在——或我們其中一位在事先準備的發言中也提到——我們在一些更急重症的服務線也看到相當健康的成長，包括心臟科、骨科與神經科。因此這也有助於急性照護的嚴重度與定價。

### **Matthew Gillmor - Equity Analyst**

Fair enough. And then maybe asking a follow-up on trends with professional fees. I heard the comments about the controlling costs, but I was curious if there's anything to report there, and give us a sense for what you're doing to try to alleviate some of that, particularly in areas like radiology.

了解。接著想追問一下專業費用 (professional fees) 的趨勢。我聽到你們提到控制成本，但我想知道這方面是否有任何可報告的情況，並請你們說明為了緩解這些壓力你們在做什麼，特別是在放射科等領域。

### **Steve Filton - Chief Financial Officer, Executive Vice President, Secretary**

Sure. So I think in our guidance for the year, we talked about professional fees rising at, I'll call it, inflationary rate in the single-digit range, maybe towards the high end of the single digits. And I think we're largely operating within that range. That's not to say that we're not seeing pressure in our markets from certain hospital-based physicians for higher fees.

當然。所以我想在我們今年的指引中，我們談到專業費用將以我稱之為通膨式的單位數增幅上升，可能接近單位數的高端。而我認為我們大致上是在這個區間內運作。這並不是說我們沒有在市場上看到某些院內醫師 (hospital-based physicians) 要求更高費用的壓力。

The way we're dealing with that is being more competitive in terms of going out for coverage with more RFPs, trying to reduce the number of locums physicians we use for our hospital-based physicians, which tend to be more expensive.

我們的因應方式是：在覆蓋/值班服務方面更具競爭力地進行更多 RFP 招標，並嘗試降低我們對臨時代班醫師 (locums physicians) 的使用量；這些臨時醫師通常成本更高，尤其是對我們的院內醫師配置而言。

And it's really just sort of a daily grind, but I think our operators have been fairly successful in keeping those professional fees at a level that is manageable, as I said, in the single digits even if it's in the high single digits.

而這確實就是一種日復一日的磨合與管理，但我認為我們的營運團隊在把專業費用控制在可管理的水準方面相當成功；如我所說，仍在單位數範圍內，即使是在單位數的高端。

## Operator

Kevin Fischbeck, Bank of America.

Kevin Fischbeck，美國銀行。

## Joanna Gajuk - Analyst

This is actually Joanna Gajuk filling for Kevin here. So maybe first, I guess, you talk about the decline in the HICS volumes, but I don't think I heard you talk about the changes in your uninsured mix. Can you talk about the payer mix a little bit more in the quarter?

其實今天是 Joanna Gajuk 代替 Kevin 發問。所以也許先問第一個：你們談到 HICS 量的下降，但我不確定是否聽到你們談到無保險組合的變化。你們能否更詳細談談本季的付款方組合？

## Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

Yes. Joanna, so what I thought I said before was, obviously, we saw a decline in HICS volumes. We saw a slight decline in Medicaid utilization, slight increase in uninsured volumes and a slight increase in Medicare volumes -- I'm sorry. (multiple speakers)

可以。Joanna，我想我先前說的是：很明顯我們看到 HICS 量下滑。我們看到 Medicaid 使用率略降、無保險量略增，以及 Medicare 量略增——抱歉。（多位講者）

## Joanna Gajuk - Analyst

(multiple speakers) Yes. No, I was just saying, so it's a slight increase in self-pay or uninsured, sorry.

（多位講者）是的。不，我只是想說，是自付（self-pay）或無保險略有增加，抱歉。

## Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

Yes.

是的。

## Joanna Gajuk - Analyst

Okay. That's helpful. And I guess, on the supplemental payment programs, right? So it sounds like there was nothing new that was approved that came through this quarter. And I guess, we're still waiting for Florida and California. So any update on these programs?

好的。這很有幫助。另外關於補充支付計畫（supplemental payment programs），對吧？聽起來本季沒有任何新核准的項目入帳。而且我想我們仍在等待佛羅里達州與加州。這些計畫有任何更新嗎？

## Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

So as far as Florida is concerned, I think -- as a number of our peers have expressed, I think there is a high level of confidence amongst providers in Florida based on feedback that we've gotten from the state that their pending 2025 program is likely to be approved. We don't know the exact timing. It could be imminent. It could be in another while.

就佛羅里達州而言，我想——如同我們一些同業所表達的——根據我們從州政府得到的回饋，佛州的提供者對其待審的 2025 年計畫很有信心，認為很可能會獲得核准。我們不知道確切時點。可能很快。也可能還要再等一段時間。

But ultimately believe the 2025 program will be approved. And when it is, we'll record it in that period and revise our guidance appropriately at that time. We have been estimating that to be about a \$50 million benefit to us. When we see the final approvals, we believe that benefit could be measurably higher, but waiting for the details from the state.

但我們最終相信 2025 年計畫會獲得核准。一旦核准，我們會在該期間入帳，並在當時適當修正我們的指引。我們一直估計這對我們約有 5,000 萬美元的利多。當我們看到最終核准內容時，我們相信這項利多可能會明顯更高，但仍在等待州政府提供細節。

As far as California is concerned, we've been asked about this before. We think that the likelihood of a renewed or expanded California program is much less likely or much less certain, I should say. And we've done nothing to try and estimate the potential benefit there until there's further, I think, sort of consensus building between the State of California and CMS. But it is possible that if there is an expanded program, it could be measurably beneficial to us, but I think it's way too preliminary to make that judgment.

至於加州，我們之前也被問過。我們認為加州計畫續期或擴大的可能性要低得多，或我應該說不確定性高得多。在加州與 CMS 之間進一步形成某種共識之前，我們沒有嘗試去估算那裡的潛在效益。但如果確實有擴大計畫，可能會對我們帶來可觀的效益；只是我認為現在下結論還為時過早。

## Operator

Justin Lake, Wolfe Research.

Justin Lake, Wolfe Research。

## Justin Lake - Analyst

Just wanted to follow up on core growth. I appreciate all the commentary about what's going to get better through the year. I think a lot of investors want to look at the hospital business right now given DPP is going to be winding down to some extent going forward on an ex DPP basis.

我想追問一下核心成長（core growth）。我很感謝你們對於今年會逐步改善的各項評論。我想很多投資人現在會關注醫院業務，因為 DPP 在未來某種程度上將逐步退場，因此大家會以剔除 DPP（ex DPP）的基礎來看。

And when I do that math, and I think about all the other moving parts, the headwinds you had on flu and weather, I get to about an \$80 million benefit on DPP plus the headwinds versus your growth of about \$50 million in the quarter on EBITDA. So it looks like core EBITDA was down \$30 million or about 5%, 6%.

當我做那個計算，並把其他變動因素都納入考量，例如流感與天氣帶來的逆風，我算出 DPP 的利多加上那些逆風合計約 8,000 萬美元，而你們本季 EBITDA 的成長約 5,000 萬美元。所以看起來核心 EBITDA 下降了 3,000 萬美元，約 5% 到 6%。

Just curious if you can help us understand, was there anything in the first quarter last year that wouldn't have recurred, for instance, any good guys that we haven't thought about? Or anything else in the core that you think was driving that minus 5%, if I'm thinking about it right?

只是好奇想請你們幫我們理解一下，去年第一季是否有任何不會再發生的項目？例如，有沒有任何我們沒想到的正面因素？或者在核心業務裡，是否還有其他你認為導致那個負 5% 的因素（如果我理解得沒錯的話）？

## Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

Yes. So, Justin, it's a little bit difficult to answer your question with precision. I don't have your calculation in front of me. But I don't dispute it. At the point that I think I made in an earlier response was simply all the points that you raised, all the points that were

raised earlier are things that we anticipated, I think that were appropriately included in our guidance.

是的。所以，Justin，要精準回答你的問題有點困難。我手邊沒有你的計算。但我不否認你的說法。我想我在先前回覆中提到的重點只是：你提出的所有點、以及先前被提出的所有點，都是我們預期會發生的，我認為也都已適當地納入我們的指引之中。

We understood that there was a difficult DPP comparison for us in the first quarter. We understood that our earnings trajectory would have to increase over the year's progression in order for us to get to that core 5% growth that's embedded in our guidance.

我們理解第一季在 DPP 上對我們而言有一個艱難的比較基期。我們也理解，為了達到我們指引中所隱含的核心 5% 成長，我們的獲利走勢必須在全年進程中逐步提升。

And for the reasons that I tried to enumerate before, I think we believe we can get there. But I acknowledge I'm not going to agree to all your numbers. I'm not suggesting they're wrong. I just don't -- I'm not able to do that off the top of my head. But I would suggest we get it that we're not at that core 5% growth, excluding all the DPP and other nonrecurring items in the quarter, but still believe that we're going to get there for the full year.

而基於我先前試著列舉的原因，我們相信我們可以達成。但我承認我不會同意你所有的數字。我不是說它們是錯的。我只是——我沒辦法在沒有資料的情況下立刻確認。但我會說，我們理解：在本季排除所有 DPP 與其他一次性項目後，我們目前還沒達到核心 5% 成長；但我們仍然相信全年會達到。

## Operator

Ben Hendrix, RBC Capital Markets.

Ben Hendrix，RBC Capital Markets。

## Ben Hendrix - Assistant Vice President

Just wanted to follow up on the HIX trends real quick. You said a 5% decline in volume, though, and then that's excluding what you expect may not actually kind of effectuate in the quarter. And I just wanted to get your thoughts on that 5% versus the 25% to 30% of HICS patients in lost coverage that you kind of embed in your assumptions for the year.

我想很快追問一下 HIX 的趨勢。你提到量下滑 5%，而且那是排除你預期可能不會在本季真正落實的部分。我想請你談談這個 5% 與你們對全年假設中「HICS 病患 25% 到 30% 失去保險覆蓋」之間的關係。

Anything changing with that or any kind of results that you saw in the first quarter that kind of informs your decision to renew that number or anything within that range for the 25% to 30% changing?

這方面有任何變化嗎？或是第一季你們看到的任何結果，是否讓你們決定維持這個數字，或在 25% 到 30% 這個區間內做調整？

## Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

Yes. So I think what we tried to articulate then was that while we could identify a 5% decline in our HICS volumes in Q1, we had an expectation that some of the patients that we were recognizing as HICS patients will later likely be identified as not having coverage because they fail to make their premium payments. So the reserve on those patients that we recorded for the first quarter reflects sort of a higher level of HICS volume decline, probably something in the low double digits, 10% or 11% or 12%.

是的。我想我們當時試著表達的是：雖然我們可以辨識出第一季 HICS 量下滑 5%，但我們預期其中一些我們目前認為 HICS 病患的人，之後很可能會被確認為沒有保險覆蓋，因為他們未能繳交保費。因此，我們第一季對這些病患所提列的準備，反映的是較高程度的 HICS 量下滑，可能是低雙位數，大約 10% 或 11% 或 12%。

We continue to believe that, that number will increase as the year goes on. Our 25% to 30% estimate for the year, we believe we may still get there. Obviously, we were not there in Q1. But again, that was an expectation that we wouldn't get there right away.

我們仍然相信，這個數字會隨著年度進行而上升。我們對全年 25% 到 30% 的估計，我們認為仍可能達到。顯然第一季我們還沒到那個程度。但同樣地，我們本來就預期不會立刻達到。

So we'll see I think what all of our peers have said and I think providers have generally said is there's still a lot of dynamics in the movement of HICS patients and who's able to make their premiums and who's not that we're going to continue to learn for at least another quarter and maybe more. But I think we feel like from an accounting perspective and reporting perspective, we're being conservative in how we're thinking about it.

所以我們會再觀察。我想我們所有同業以及醫療服務提供者普遍都表示，HICS 病患的流動、以及誰能繳保費、誰不能繳之間仍有很多動態，我們至少還會再學習一個季度，甚至更久。但我認為，從會計與揭露角度來看，我們在思考這件事的方式是偏保守的。

## Operator

Andrew Mok, Barclays.

Andrew Mok, Barclays。

## Andrew Mok - Analyst

Given that flu and weather were early quarter dynamics, can you comment on the progression of volumes throughout the quarter in each segment, including exit rates in March and April?

鑑於流感與天氣是季初的動態因素，你能否評論一下本季各事業部的量在季度內的變化走勢，包括 3 月與 4 月的退出（期末）水準？

## Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

Well, what I would say, Andrew, is I think the flu volume comparison was more significant in January and February. I think flu season was largely over last year and this year as we got to the end of the quarter. So I think that was -- the significant sort of flu decline was felt more acutely in the first couple of months of the quarter.

Andrew，我會說，我認為流感量的比較在 1 月與 2 月更為顯著。我想去年與今年的流感季在接近季末時大致都已結束。所以我認為——顯著的流感下滑在本季前兩個月感受更為明顯。

And then, the weather-related, really depended on the market, but we had storms in both January and February that affected, I think, both business segments. Again, I think March was, for want of a better word, a cleaner month, meaning no real flu impact and no significant weather impact. And I think March volumes were -- showed a more normative increase over last year.

至於天氣相關影響，確實取決於市場，但我們在 1 月與 2 月都有風暴，我想影響了兩個業務部門。同樣地，我認為 3 月用比較貼切的說法是「更乾淨」的一個月，也就是沒有真正的流感影響，也沒有顯著的天氣影響。我認為 3 月的量——相較去年呈現較為常態化的增長。

## Operator

Raj Kumar, Stephens.

Raj Kumar, Stephens。

## Raj Kumar - Equity Analyst



Maybe following up on the kind of March, April trends. I guess -- did you see a significant pickup in potential kind of deferred care that got pushed back because of the winter storms? And then maybe just also any kind of commentary on acute care surgical volume trends and just general acuity profile shifts year-over-year?

也許延續一下 3 月、4 月的趨勢。我想問——你們是否看到因冬季風暴而被延後的醫療需求，在之後有明顯回補？另外也請評論一下急性照護的外科手術量趨勢，以及整體病情嚴重度（acuity）輪廓在年對年上的變化？

### **Steve Filton - Chief Financial Officer, Executive Vice President, Secretary**

Well, what I would say generally is that what we find are elective procedures, and that's mostly in the acute segment that are scheduled and postponed because of the weather, tend to be rescheduled. But as we indicated, the bigger impact in acute care was the flu, which is not something you recover from. So I think we talk about maybe \$5 million to \$7 million of weather impact. And that's mostly in the D.C. market, where we had some burst pipes in one of our facilities and had to close beds for a period of time.

我一般會說，我們觀察到的是：因天氣而被排程、被延後的選擇性手術（多半在急性照護部門）通常會被重新安排。但如同我們所指出的，急性照護更大的影響是流感，而那不是你「補回來」的。所以我想我們提到天氣影響大約 500 萬到 700 萬美元。而那主要發生在華盛頓特區市場，我們其中一個設施有水管爆裂，必須在一段時間內關閉病床。

Quite frankly, that's not something you can really recover from, the bed closure. And on the behavioral side, same thing. You'll recover outpatient visits, et cetera, but inpatient trauma sorts of admissions are generally going to go somewhere else if they can't get to the hospital.

坦白說，病床關閉這件事你很難真正補回來。在行為健康（behavioral）方面也是一樣。你可能會補回門診就診等，但住院的創傷類入院通常會在無法到院時轉往其他地方。

So yes, again, I think that's something that we're not -- as I talk about, our recovery for the rest of the year or our growth for the rest of the year, we're not really counting on a tremendous amount of recapture of those deferred procedures.

所以是的，再次強調，我認為這也是為什麼我在談我們今年剩餘時間的復甦或成長時，我們並沒有把大量回補那些被延後手術的再捕捉（recapture）納入假設。

### **Operator**

Craig Hettenbach, Morgan Stanley.

Craig Hettenbach, Morgan Stanley。

### **Craig Hettenbach - Analyst**

I wanted to go back to the strategy in outpatient behavioral health and the Talkspace deal. Can you maybe just touch on kind of 1,000 Branches? How that's been going? And how much that kind of informed this next step on Talkspace?

我想回到門診行為健康的策略以及 Talkspace 這筆交易。你能否談一下 1,000 Branches？目前進展如何？以及它在多大程度上促成了你們在 Talkspace 上的下一步？

### **Marc Miller - President, Chief Executive Officer, Director**

Sure. I mean, things are going well. It's been a little bit slower deployment than we would have thought just with various different situations state by state. But it didn't really inform us at all as far as being an impetus for Talkspace. We've known about Talkspace for many years. We obviously follow the landscape.

當然。我的意思是，進展良好。只是因各州不同情況，推進速度比我們原先想的稍慢一些。但就 Talkspace 而言，它其實完全沒有影響我們、也不是促成 Talkspace 的動因。我們認識 Talkspace 很多年了。我們當然也一直在追蹤整個市場版圖。

So we know everybody that's out there. And we've talked a lot about our own outpatient deployment for many years now and wanting to become more proficient in outpatient. So 1,000 Branches is something we've been working on for a number of years.

所以我們知道市面上所有的參與者。而且我們多年來一直在談我們自己的門診佈建，以及希望在門診領域變得更熟練。因此，1,000 Branches 是我們已經投入推動多年的事情。

But Talkspace happened because there was an opportunity that we didn't really create. And once that company indicated that they were interested in at least considering options, we began discussions and it got to where it got to.

但 Talkspace 的發生，是因為出現了一個我們其實並未真正創造的機會。而一旦那家公司表示他們至少有興趣考慮各種選項，我們就開始討論，最後就發展到現在這個結果。

So I wouldn't say that one led us to the other. I would say that we've been studying that landscape for a long time. And we were pleased, and we are pleased, that we're able to go in both directions because I think that our current outpatient offerings and the ones that we're developing ourselves internally are only going to help us, as we combine with Talkspace and grow, as I said earlier, our whole continuum.

所以我不會說其中一個促成了另一個。我會說，我們研究那個市場版圖已經很長一段時間了。而我們很高興、也仍然很高興，我們能夠兩個方向都能推進，因為我認為我們目前的門診產品，以及我們在內部自行開發的那些，只會在我們與 Talkspace 結合並成長、如我先前所說擴展我們整個照護連續體時，對我們更有幫助。

### **Craig Hettenbach - Analyst**

Got it. And then just as a quick comment, you mentioned kind of the effective multiple to be in single digits a few years out. Just what gives you that confidence in terms of what you're seeing in their business? And what you can do with it to drive that effect of multiple down?

了解。然後快速追問一下，你提到幾年後有效倍數會落在個位數。就你從他們的業務中看到的情況而言，究竟是什麼讓你有這樣的信心？以及你們能如何運用它來推動倍數下降到那個程度？

### **Marc Miller - President, Chief Executive Officer, Director**

Yes. I mean, you said it actually in the question. I mean, we have confidence now that we've had a full look at their business model, what they achieved just in the last couple of years and what their plans are, and we think they can achieve. I'll even say what they were going to achieve on their own in the next 24 months. Add that to what we think we can do to help them increase those achievements and earnings.

是的。我的意思是，你其實在問題裡就說到了。我的意思是，我們現在有信心，因為我們已經完整檢視了他們的商業模式、他們在過去幾年所達成的成果，以及他們的計畫，並且我們認為他們能夠達成。我甚至可以說，他們在未來 24 個月內即使靠自己也會達成的成果。再加上我們認為我們能做的事情，來幫助他們提升這些成果與獲利。

That's what gives us greater confidence that a few years from now, we'll look back on this, and I think the multiple will be single digits because when you look at it now, we've had folks ask us about it. It's harder to understand the multiple if you just look at it from today's earnings. But in the next couple of years, again, they're on a great growth path themselves, and we're going to add to that once we combine resources.

這就是讓我們更有信心的原因：幾年後回頭看這筆交易時，我認為倍數會是個位數，因為如果你現在看，確實有人問過我們。如果只從今天的獲利來看，倍數比較難理解。但在接下來幾年，他們本身就處在很好的成長軌道上，而我們在整合資源後也會在此基礎

上再加碼。

## Operator

Benjamin Rossi, JPMorgan.

Benjamin Rossi，摩根大通。

## Benjamin Rossi - Analyst

Just following up on the volume discussion across segments. Now that we're through the first quarter, what's your current outlook for Medicaid volumes for the year for both segments? And are you seeing any signs of volatility returning either through administrative churn or eligibility friction to start the year?

接續追問各業務分部的量能討論。現在第一季已經結束，你們目前對今年兩個分部的 Medicaid 量的展望是什麼？以及你們是否看到年初因行政性流失（administrative churn）或資格認定摩擦（eligibility friction）而使波動回升的任何跡象？

## Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

Yes. Ben, I mean, yes, as I've now said a couple of times on the call, we saw, I think, slight declines in Medicaid utilization in Q1. I think that's reasonably consistent with our expectations for the year. We're not expecting any other major changes, either in Medicaid, quite frankly, or in payer mix other than the HICS commentary that we've talked about a couple of times.

是的。Ben，我的意思是，是的，正如我在電話會議中已經說了幾次，我們在第一季看到 Medicaid 使用量略有下降。我認為這與我們對全年預期大致一致。我們不預期還會有其他重大變化，坦白說，不論是在 Medicaid 或在付款方組合（payer mix）方面，除了我們已經談過幾次的 HICS 相關評論之外。

So yes, again, I would describe the changes to payer mix outside of HICS in Q1 as relatively minor and pretty consistent with our expectations. And I think we think about that as being -- likely to be true for the rest of the year as well.

所以是的，再次強調，我會把第一季在 HICS 以外的付款方組合變化描述為相對輕微，且與我們的預期非常一致。而我們也認為今年剩餘時間大概率也會如此。

## Benjamin Rossi - Analyst

Okay. Just then -- as a follow-up then on the de novo side, can you just provide an update on your de novo hospitals? And how you're thinking about full year EBITDA performance across some of your recent openings in Nevada and D.C. and along with the Florida Hospital set to open up next month?

好的。那接著追問 de novo 方面，你能否更新一下你們的 de novo 醫院狀況？以及你們如何看待全年 EBITDA 表現，涵蓋你們近期在內華達州與華盛頓特區的新開院所，並加上預計下個月開幕的佛羅里達醫院？

## Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

Yes. I'll just -- I'll remind people that as part of our guidance, we talked about the idea that the new hospital in Florida, which is really the true de novo for this year, would likely experience an operating loss for the year as with most new hospitals. We believe, and our guidance contemplates that, that whatever loss that it incurs would likely be largely offset by gains and improvements at our Cedar Hill facility in Washington, D.C.

是的。我先提醒大家，作為我們指引的一部分，我們談到今年佛羅里達的新醫院——這其實才是今年真正的 de novo——很可能會像大多數新醫院一樣，全年出現營運虧損。我們相信，而且我們的指引也已納入這點：它產生的任何虧損，很可能會在很大程度上被我們位於華盛頓特區 Cedar Hill 院區的收益與改善所抵銷。

I think we would reiterate that guidance, although I think we are of the mind that the Cedar Hill improvement is likely more back-end loaded maybe than we originally contemplated. It's gotten -- it's had some issues with the weather and some of the other dynamics that we've talked about.

我想我們會重申該指引，不過我認為我們目前的看法是，Cedar Hill 的改善可能比我們原先預期更偏向在下半年才會顯現。它受到天氣以及我們談過的一些其他動態因素影響，出現了一些問題。

The other new capacity that we referred to in our prepared remarks, new towers in Las Vegas and in the West Coast of Florida and a replacement facility in California, these are in existing markets, and they'll all come online at some point during the second quarter, and we expect they'll ramp up relatively quickly because they're in existing markets and essentially just adding to our existing operations. So we believe that they will have a very positive impact on growth in the back half of the year.

我們在事先準備的發言中提到的其他新增產能——拉斯維加斯的新大樓、佛羅里達西岸的新大樓，以及加州的替換院區——這些都在既有市場中，並且都會在第二季某個時間點上線；我們預期它們會相對快速爬坡，因為它們位於既有市場，本質上只是擴充我們既有營運。因此我們相信，它們將在下半年對成長帶來非常正面的影響。

## Operator

Ryan Langston, TD Cowen.

Ryan Langston, TD Cowen。

## Ryan Langston - Analyst

I just wanted to follow up and make sure I understood your comments on the denials activity. Are you saying you are seeing accelerating levels of denials, but you're just able to navigate them more effectively? And are you able to provide any further details on what you're seeing by payer clients?

我想追問並確認我是否理解你們對拒付（denials）活動的評論。你們是說拒付水準正在加速上升，但你們只是能更有效地因應嗎？另外，你們能否就不同付款方客戶（payer clients）所呈現的情況提供更多細節？

## Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

So -- no, I don't think we're seeing an increased level of denials, Ryan, of any material amount. What I suggested I think is that because I think others have talked about maybe more aggressive behavior on the part of payers. And all I was trying to make the point that I think the investments that we've made in our own revenue cycle, particularly in the acute space, both from a technology perspective, but also from personnel and process perspective are allowing us to sort of keep pace with potentially more aggressive behavior on the part of the payers. And I've just simply also made the point that we expect to make similar investments over the next 12 to 18 months in our behavioral health revenue cycle functions.

所以——不，我不認為我們看到拒付水準有任何實質幅度的上升，Ryan。我想我所表達的是，因為我認為其他人可能談到付款方行為變得更具侵略性。而我只是想指出，我們在自身收入循環（revenue cycle）上的投資，特別是在急性照護領域，無論是技術面，還是人員與流程面，都讓我們能夠跟上付款方可能更具侵略性的行為。我也只是補充一點：我們預期在未來 12 到 18 個月內，會在行為健康（behavioral health）的收入循環職能上進行類似投資。

## Operator

Scott Fidel, Goldman Sachs.

Scott Fidel, 高盛。

## Scott Fidel - Analyst

First question, would be interested if you could give us an update on just how you're seeing the sort of underlying trend in behavioral in terms of just the demand versus supply equilibrium, if there's -- relatively consistent with where we've been the last couple of years or just any type of incremental shifts you may be seeing on -- either on the supply or the demand side?

第一個問題，我想知道你們對行為健康領域的基本趨勢有何更新：就需求與供給的均衡而言，是否大致與過去幾年一致，或你們是否看到任何增量變化——不論是在供給端或需求端？

### **Steve Filton - Chief Financial Officer, Executive Vice President, Secretary**

Scott, in that regard, I think we've been pretty consistent for several years now in suggesting that we think behavioral demand remains strong and probably our greatest challenge is then in meeting that demand. Probably the single biggest obstacle we've had in meeting that demand for the last several years has been inadequate staffing in certain markets, in certain personnel functions, sometimes that's nurses, sometimes that's therapists, sometimes that's mental health technicians, the unlicensed personnel in our behavioral facilities.

Scott，就這點而言，我認為我們多年來一直相當一致地表示，我們認為行為健康的需求仍然強勁，而我們最大的挑戰大概是如何滿足這個需求。過去幾年在滿足需求方面最大的單一障礙，可能是在某些市場、某些職能的人力不足；有時是護理師，有時是治療師，有時是心理健康技術員，也就是我們行為健康機構中的非執照人員。

The only other, I think, comment that we've made about demand, particularly, I would say, in the last several quarters or maybe the last year, is that while the demand remains strong, I think we see it as shifting, not unlike the way it has shifted in the acute segment for a decade or more at this point to more outpatient delivery.

另外一點我想補充、我們也談過的需求面觀察，特別是在最近幾季或過去一年：雖然需求仍然強勁，但我們認為它正在轉移；這與急性照護分部在過去十多年所出現的轉移類似，正逐步轉向更多門診交付。

And as Marc alluded to in his previous answer, we're trying to meet that demand in a number of different ways, building out our own freestanding -- ability to deliver care in freestanding facilities with our 1,000 Branches initiative, more focused on the step down of patients who are being discharged from our existing facilities, and obviously, through the Talkspace acquisition as well. So strong demand, although to a degree that demand is shifting more and more to the outpatient setting.

而如 Marc 在前一個回答中提到的，我們正透過多種方式來滿足該需求：推進我們自己的獨立式（freestanding）照護交付能力，透過我們的 1,000 Branches 計畫，更聚焦於承接從既有院區出院的病患之下轉（step down）照護；當然也包括透過收購 Talkspace。所以需求強勁，只是其中一部分需求正愈來愈轉向門診場域。

### **Scott Fidel - Analyst**

Okay. So if I could ask a follow-up around that, Steve, and around the outpatient strategy that's underway. And particularly, just as it relates to how that's informing your thinking right now on capital allocation. And specifically obviously, you have the outpatient clinics expansion going on, you have the Talkspace acquisition.

好的。那麼如果我可以就此追問一下，Steve，並且就目前正在推進的門診策略。尤其是，這如何影響你目前對資本配置的思考。具體來說，顯然你們正在擴張門診診所，同時也進行了 Talkspace 的收購。

I'm curious around how you're weighing sort of balancing buybacks relative to sort of continuing to invest more, allocating more capital towards continuing to build out that full capacity flywheel on the outpatient and digital side?

我想知道你們如何權衡：在持續加大投資、把更多資本配置到門診與數位端、持續打造完整產能飛輪的同時，相對於股票回購之間的平衡？

### **Steve Filton - Chief Financial Officer, Executive Vice President, Secretary**

Well, I think the point that we've tried to make since announcing the Talkspace acquisition, we had a number of questions, I think, when we first announced sort of in the context of, well, this is a fairly large acquisition, how does -- is this an either/or in terms of share repurchase. And I think, again, as we tried to articulate in our remarks, we continue to find buyback of our own shares to be a pretty compelling investment and intend to remain active in that regard.

嗯，我想自從我們宣布收購 Talkspace 以來，我們一直試圖傳達的一點是：我們收到不少問題，我想在最初宣布時，大家在某種程度上會問，在「這是一筆相當大的收購」的背景下，這是否——在股票回購方面是一個二選一的取捨。而我想，再次如同我們在發言中試圖闡述的，我們仍然認為回購自家股票是一項相當具吸引力的投資，並打算在這方面保持積極。

I think we talked on our end-of-the-year call as having a buyback target in the \$800 million to \$900 million range, and I think that certainly remains a target for us at a minimum. The Talkspace acquisition does not increase our leverage significantly. We go from a little under 2 times levered to a little over 2 times, but certainly, that leaves us plenty of space to consider, as we said in our prepared remarks, any other acquisitions that would come up that we found compelling as well as a continued aggressive CapEx program as well as continue being an active return of capital to our shareholders.

我想我們在年終電話會議上提到，我們的回購目標在 8 億到 9 億美元的區間，而我認為這至少仍然是我們的目標。Talkspace 的收購並未顯著提高我們的槓桿。我們從略低於 2 倍槓桿上升到略高於 2 倍，但這仍然為我們留下充足空間，去考慮——正如我們在事先準備的發言中所說——任何其他我們認為具吸引力的收購機會，同時也能持續推進積極的資本支出（CapEx）計畫，並且繼續積極向股東回饋資本。

## Operator

Michael Ha, Baird.

Michael Ha, Baird。

## Michael Ha - Senior Research Analyst

Just quickly first following up on Justin's question. I understand you're not providing the core growth math at the moment, but I wanted to ask you about the inputs and to see if I'm missing any major components of this math. So far, I'm looking at net DPPs, the exchange subsidy headwind, California staffing requirement headwind, the flu weather impact.

先快速跟進 Justin 的問題。我理解你們目前不提供核心成長的計算方式，但我想問一下其中的輸入項，看看我是否漏掉了這個計算中的任何主要組成。到目前為止，我看到的是淨 DPP、交易所補貼的不利因素、加州人力配置要求的不利因素、流感與天氣的影響。

But then, how should we treat any Palm Beach Gardens de novo costs, the Cedar Hill tailwind, which sounds more back-end loaded now? And is there anything else that we should consider, anything on first quarter last year that would not recur?

那麼，Palm Beach Gardens 新設（de novo）成本應該如何處理？以及 Cedar Hill 的利多——聽起來現在更偏向在後段發生？另外還有沒有其他需要考量的因素？例如去年第一季是否有任何不會再發生的項目？

## Steve Filton - Chief Financial Officer, Executive Vice President, Secretary

Yes. I mean, everything that you ticked through, Michael, are I believe items that we've discussed already. Again, I made the point earlier, and I'll just reiterate, none of those items were really a surprise to us during the quarter, and our overall results in the quarter were very consistent with our internal expectations.

是的。我的意思是，你逐一系列出的那些，Michael，我相信都是我們已經討論過的項目。再次強調我先前提到的一點：這些項目在本季對我們而言都不算意外，而我們本季的整體結果也與我們內部預期非常一致。

I think these were all knowns going into the quarter, obviously, other than the weather and the flu, although we certainly, I think, referenced both on our fourth quarter call. Your question about Cedar Hill and the new hospital in Florida, again, I'll reiterate, we said in our guidance, we thought that would be a wash for the year largely. That continues to be our belief, although I made the point that I think the Cedar Hill improvement is probably a little bit back-end loaded.

我認為這些在進入本季之前都是已知事項，當然，天氣與流感除外，不過我想我們在第四季電話會議上也確實提到過這兩點。至於你問到 Cedar Hill 與佛州新醫院的問題，我再重申一次：我們在指引中表示，我們認為就全年而言，這兩者大致會互相抵銷、影響為零。我們仍然這麼認為，雖然我也提到，我認為 Cedar Hill 的改善可能會比較偏向在後段發生。

But -- no, I mean, I think we've covered all those items. Those were all well within our expectations, all included in our guidance.

但是——不，我的意思是，我想我們已經涵蓋了所有這些項目。這些都完全在我們的預期之內，也都已納入我們的指引。

### **Michael Ha - Senior Research Analyst**

Got it. And then, following up on Scott's question on behavioral health, I know you had implemented labor efforts focused on retention at year one hires since the turnover rate of post year one hires diminishes drastically. Wondering how those efforts gone? I know last time we spoke, the turnover rate was moving from as high as 50% down to at least 40% over the last half year.

了解。接著跟進 Scott 關於行為健康（behavioral health）的問題，我知道你們已推行一些以留任為重點的人力措施，聚焦在入職滿一年的員工，因為滿一年後的離職率會大幅下降。想請問這些措施進展如何？我記得上次我們談到時，離職率在過去半年從最高約 50% 降到至少 40%。

Curious where are you today on that turnover rate? Is it still in the 40s? And if you could remind us what your pre-COVID turnover rate was just to establish reference point, that would be great.

想了解你們目前的離職率是多少？是否仍在 40% 多的區間？另外，如果你能提醒我們 COVID 之前的離職率是多少，作為參考基準，那就太好了。

### **Steve Filton - Chief Financial Officer, Executive Vice President, Secretary**

Yes. So again, I think, as we said in our prepared remarks, I reminded people that last year, our salary and wage expense in behavioral was up 8%, which included a pretty significant increase in the headcount. In Q1, it was more in the 6% to 7% range. So we saw some moderation in Q1. We expect to see further moderation as the year goes on.

是的。所以再次說明，我想如同我們在事先準備的發言中所說，我提醒大家，去年我們在行為健康業務的薪資與工資費用增加了 8%，其中包含人數相當顯著的增加。第一季則大約在 6% 到 7% 的區間。所以我們在第一季看到一些趨緩。我們預期隨著年度推進，還會看到進一步的趨緩。

That's a function, I think, of any number of things. It is -- we're not hiring as aggressively in '26 as we did in '25, that's number one. Number two, I think we're seeing some moderation in wage increases. And again, I think we expect and as your question implied, we're also seeing some progress on turnover. Turnover remains high in behavioral. It remains high, I think, in all sub-acute industries, but we have clearly made progress and measurable progress on turnover in the last year or so.

我認為這是多種因素共同作用的結果。第一，我們在 26 年的招募力度不會像 25 年那麼積極。第二，我想我們也看到薪資增幅有所趨緩。另外，正如你的問題所暗示的，我們也在離職率方面看到一些進展。行為健康的離職率仍然偏高。我認為在所有亞急性（sub-acute）產業中離職率都偏高，但我們在過去一年左右確實已經取得明顯且可衡量的改善。

### **Operator**

I'm showing no further questions at this time. I would now like to turn it back to Darren Lehigh for closing remarks.

目前顯示沒有進一步的問題。我現在想把電話交回給 Darren Lehrich 作結語。

Darren Lehrich - Vice President of Investor Relations

Thank you, everyone, for participating in today's call and for your interest in UHS. Have a great rest of your day.

感謝各位參與今天的電話會議，也感謝各位對 UHS 的關注。祝各位今天剩下的時間愉快。

Operator

This concludes today's conference call. Thank you for participating. You may now disconnect.

今天的電話會議到此結束。感謝各位參與。您現在可以斷線。

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